

How to Track Pain for Doctors

A short guide to what clinicians actually need from your record and how to stop handing them a maze.

Routing Note

- Need:** Doctors do not need more generic content. They need sharper routing through the facts that matter.
- Tool:** Use this guide when you want your notes to stop sounding scattered and start acting like a triage system.
- Rule:** Pick the need. Hand over the pattern. Move the conversation one step closer to control.

What Doctors Need to See

1. Pattern Before Performance

Doctors do not need a dramatic monologue. They need a pattern they can trust. Bring at least a week of dated entries so the visit starts with evidence instead of reconstruction.

2. Sharp Description Beats Big Description

Where exactly? What kind of pain? When did it start, stop, move, or spike? Sharper routing through the facts is better than trying to make the pain sound impressive.

3. Function Is the Lever

"Could not finish groceries" or "needed help showering" carries more weight than a floating number. Pain becomes clinically useful when it is tied to what the body could not do.

4. Treatment Response Needs a Trail

What did you take, when did you take it, what changed, and what did it cost? Relief without timing is weak. Side effects without context are weak. The chain matters.

5. Sleep, Mood, and Pain Are One System

Pain does not stay politely in one category. Sleep damage, mood strain, and cognitive drag belong in the record because they change treatment decisions.

Appointment Preparation Worksheet

Appointment date: _____

Doctor/Specialist: _____

Main concerns to discuss: _____

Changes since last visit: _____

Questions to ask: _____

Current medications: _____

Appointment Notes